DECISION INTELLIGENCE · HEALTH, LIFE SCIENCES & CARE SYSTEMS

The Summer Medicine Chose Sides

Edition 2 in the Health, Life Sciences and Care Systems Decision Intelligence series. Previous edition: ‘Too Much Cure, Not Enough Care’ (June 2026). Topic cycle drawing on ~40 sources within a 180-day window.

Cycle: 6 August 2026 · Audience: Health, life-sciences and care-system leadership · Horizon: to 2030

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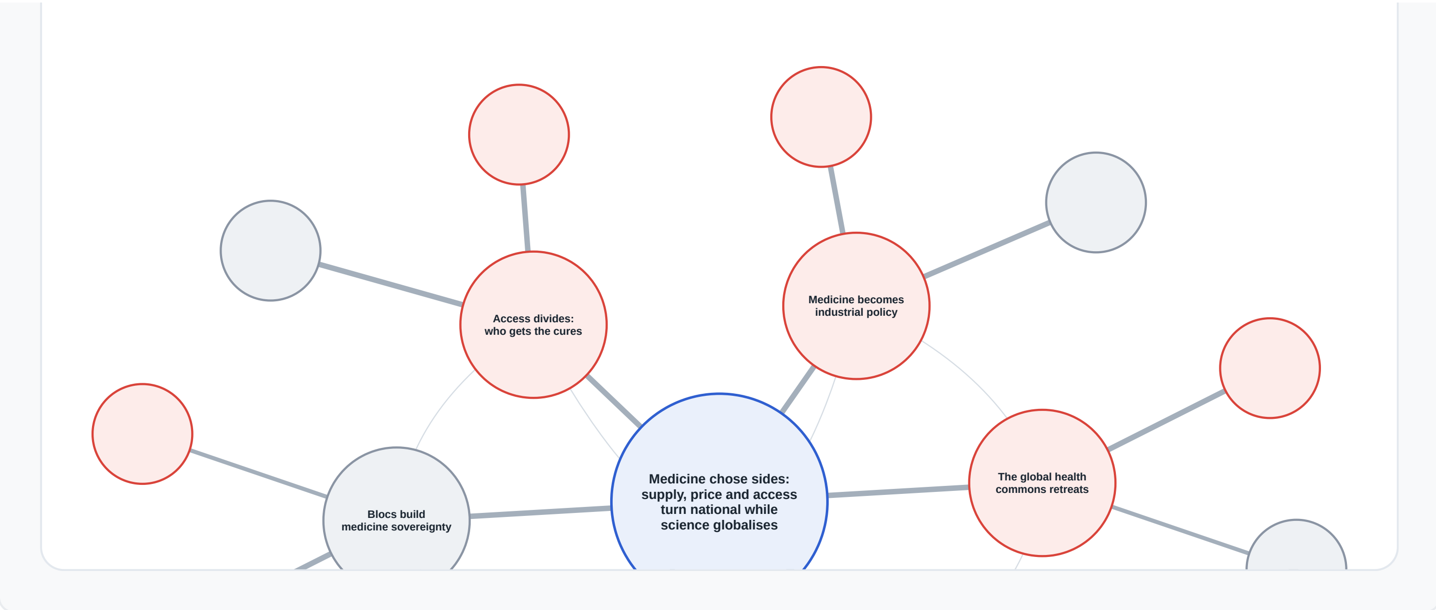
Contents

risk opportunity mixed/neutral impact × likelihood: high med low



What you'll learn

1. Why medicine became industrial policy in 2026: 100 percent US pharmaceutical tariffs, seventeen most-favored-nation pricing deals, and the EU's Critical Medicines Act turn supply and price into instruments of state power.
2. Why the global health commons is retreating: the completed US exit from the WHO, deep aid cuts, and a Global Fund replenishment billions short leave a financing vacuum that others are scrambling to fill.
3. Why discovery and access are accelerating even as the politics fragment, with in-vivo gene editing, the first oral GLP-1, expanded CRISPR access and joint FDA and EMA principles for AI in drug development.
4. Why care is going AI-native, as ambient scribes move from a few percent of visits toward a third, roughly three-quarters of health systems adopt AI, and the safety and evidence gaps follow close behind.

Key takeaways

1. On 2 April 2026 the US imposed 100 percent Section 232 tariffs on patented pharmaceuticals, effective 31 July, while seventeen manufacturers representing 86 percent of the branded market signed most-favored-nation pricing deals, and the EU reached a Critical Medicines Act agreement on 12 May with a Buy-European procurement tilt.
2. The US completed its WHO withdrawal on 22 January 2026; the WHO's 2026-27 budget was cut 21 percent to 4.2 billion dollars, the Global Fund's replenishment fell 5.4 billion dollars short of target, and international HIV financing dropped 1.5 billion dollars in a year, with more than three million new infections projected by 2030.
3. The science accelerated regardless: an in-vivo base editor cut LDL cholesterol 52 percent, the FDA approved the first oral GLP-1 and expanded the CRISPR therapy Casgevy to children as young as two, and the FDA and EMA published joint principles for AI in drug development.
4. Care went ambient: one large deployment saw AI-scribe use rise from under 3 percent to about 31 percent of visits, roughly 75 percent of US health systems now use or plan to use an AI platform, yet 4.8 percent of AI-enabled devices have been recalled and the FDA now lists about 1,450 of them.

Executive Synthesis

Why this matters now

This was the summer medicine chose sides. Within a few months the US put a 100 percent tariff on patented drugs and tied it to price, seventeen manufacturers signed pricing deals, the EU passed a Critical Medicines Act with a Buy-European tilt, and the US finished walking out of the World Health Organization it once built. Supply, price and access, the three things that decide who gets which medicine, became instruments of statecraft. And yet in the same months an in-vivo gene editor rewrote cholesterol in a single dose, the first oral GLP-1 was approved, and AI moved from the lab bench to the consulting room. The pattern this cycle records is a split screen: the politics of medicine fragmenting into national blocs while the science and the tools converge and accelerate, and access dividing between a rich world of abundance and a poor world in retreat.

Four shifts this cycle records, and the pattern that names them:

- **Medicine becomes industrial policy.** Tariffs, most-favored-nation pricing and the EU's Critical Medicines Act turn drug supply and price into instruments of national power.
- **The global health commons retreats.** The US WHO exit, collapsing donor finance and a short Global Fund leave a governance and financing vacuum others scramble to fill.
- **Discovery and access accelerate.** In-vivo gene editing, an oral GLP-1, expanded CRISPR access and joint AI principles widen what medicine can do, and reopen the fight over who can afford it.
- **Digital health goes ambient.** AI scribes and agentic workflows move into the consulting room faster than the evidence and safety base behind them.

This cycle names the variable that will decide the shape of health to 2030: not whether the science advances, that is settled, but whether the politics of medicine re-integrate or stay fragmented, and whether the new cures reach broadly or are captured narrowly. Four developments carry the pattern.

First, medicine became industrial policy. On 2 April 2026 the US imposed a 100 percent Section 232 tariff on patented pharmaceuticals, effective 31 July for the largest firms, with generics and orphan drugs exempt (Arnall Golden Gregory). The tariff is a lever, not a wall: the White House tied it to price, offering zero tariffs to companies that sign most-favored-nation pricing agreements, and by late April seventeen manufacturers covering 86 percent of the branded market had signed. Analysts question the projected savings and warn tariffs could raise US drug prices by roughly 30 percent. Europe answered in kind: on 12 May the EU reached a provisional Critical Medicines Act with a Buy-European procurement tilt and strategic manufacturing projects (RAPS). The result is a scramble for supply security, from 480 billion dollars of US reshoring pledges to India's persistent dependence on Chinese ingredients.

Second, the global health commons retreated. The US completed its WHO withdrawal on 22 January 2026, and the institution's 2026-27 budget was cut 21 percent to 4.2 billion dollars with a tenth of staff gone. Aid followed: the OECD projects a 6.9 percent fall in global aid in 2026, the Global Fund's replenishment fell 5.4 billion dollars short as France cut its pledge, Gavi came in below target with US money withheld, and international HIV financing dropped 1.5 billion dollars in a year, with more than three million new infections projected by 2030. The vacuum is being contested rather than filled: China is now the WHO's top assessed contributor but a minor voluntary donor, and African leaders are talking openly of health sovereignty as combined US and Global Fund cuts land.

Third, discovery and access accelerated, indifferent to the politics. A single dose of an in-vivo base editor cut LDL cholesterol 52 percent in a phase 1 trial, and a rival programme cut it 62 percent; the FDA expanded the CRISPR therapy Casgevy to children as young as two. The FDA approved the first oral GLP-1, Foundayo, cleared by letter on 1 April 2026, and CMS built a Medicare GLP-1 Bridge through its BALANCE model. On method, the FDA and EMA issued joint principles for AI in drug development, regulators are rethinking what counts as a breakthrough device, and observers argue approval of an AI-designed drug is not far off. The science does not always deliver: a GLP-1 failed to slow Alzheimer's disease.

Fourth, care went ambient. Ambient AI scribes crossed from novelty to infrastructure: one Spanish deployment saw adoption rise from 2.7 percent to about 31 percent of outpatient visits across 2.3 million encounters, a controlled study found scribes cut documentation time and lifted eye contact, and US systems report burnout and note-time reductions (AHA). About 75 percent of US health systems now use or plan to use an AI platform, Epic previewed an agent-building Factory, and Abridge partnered with Nvidia and Lilly to move beyond documentation. Consumers followed, with one in three US adults now using AI for health questions. But the evidence and safety base lags: the FDA lists about 1,450 AI-enabled devices, 4.8 percent of them recalled, even as physicians rate most AI-generated summaries as low-harm.

The science of medicine has never been more universal, and its politics never more national; the gap between the two is where the next decade of health will be decided.

The rhyme worth holding is energy after 1973. Oil had been a traded commodity; the shock turned it into an instrument of national security, and for fifty years states have subsidised, stockpiled and reshored it, accepting higher cost for sovereignty. Medicine is now taking that path. Tariffs, local-content rules and strategic-manufacturing designations will make supply more resilient and more expensive, and, as with energy, the countries that can pay will secure their own while the poorest are left most exposed. The prudent posture is to read tariffs and pricing, the governance retreat, the discovery wave and ambient AI as one connected story about where medicine is made, who pays for it and who is left out, not four separate headlines.

Where this analysis could be wrong

The most consequential assumption is that fragmentation persists and hardens. If the tariff regime proves to be a negotiating lever that is largely unwound through pricing deals, and if a change of US administration or a public-health emergency pulls Washington back toward the multilateral system, the "sides" of 2026 could soften within the horizon. The falsifiable tests: whether the 100 percent tariffs are actually collected in 2027 or serially waived; whether the US resumes any WHO or Gavi funding before 2028; and whether the EU's Buy-European rules survive contact with member-state budgets. Signals to watch: the first enforced tariff collection or a broad waiver, a US re-engagement with any multilateral health body, and whether reshoring pledges convert into operating plants.

What this cycle establishes

1. **Medicine as industrial policy.** Tariffs, MFN pricing and the Critical Medicines Act are in force or agreed. The open variable is whether they are collected and sustained, or unwound through deals.
2. **The commons in retreat.** The US exit and the financing shortfall are facts. The open variable is who fills the vacuum, and whether a multipolar order stabilises or frays.
3. **Discovery accelerating.** Gene editing, oral GLP-1s and AI methods are advancing fast. The open variable is whether access broadens through coverage or narrows to

those who can pay.
4. **Care going ambient.** AI is in the consulting room now. The open variable is whether the safety, evidence and oversight base catches up before an adverse event.
5. **Name the pattern: medicine chose sides.** The politics fragment while the science converges, so the decisive question shifts from what is possible to who provisions it, and for whom.

Each is developed below, with a decision posture, in the four Strategic Implications.

Continuity ledger: how the June cycle's six forces have moved

The June edition, “Too Much Cure, Not Enough Care”, named six forces. One has escalated, one is confirmed, one is recast, two carry forward, and one, the expectation of regulatory convergence, has reversed into the fragmentation this cycle records.

June force	Status	August read
Drug Pricing Pressure Accelerates in 2026	ESCALATED	Pricing pressure became full industrial policy: a 100 percent US tariff on patented drugs tied to most-favored-nation pricing deals.
Regulatory Convergence Across Major Markets	REVERSED	The opposite happened. Markets diverged: US tariffs and MFN pricing, an EU Buy-European Critical Medicines Act, and the completed US exit from the WHO.
GLP-1s Reshape Metabolic Disease Economics	CONFIRMED	Confirmed and widening: the first oral GLP-1 was approved and a Medicare GLP-1 Bridge launched.
AI Diagnostics from Authorisation to Reimbursement	RECAST	Recast and broadened into ambient and agentic AI in care: about 75 percent of systems now adopt AI, but 4.8 percent of AI devices have been recalled.
The Ageing-Workforce Inflection	CARRIED	Carried forward, not re-examined this cycle; care-system strain compounds as global health aid falls.
Mental Health Crisis Meets Digital Therapeutics	CARRIED	Carried forward; the digital-care momentum continues, with one in three US adults now using AI for health questions.

Audience Snapshots

Four functional lenses on the same intelligence base. Each card names the shift and the single question this cycle puts to that part of the sector.

PHARMA & BIOTECH LEADERSHIP

Is your supply and pricing model built for a tariff-and-sovereignty world?

The shift: a 100 percent US tariff on patented drugs is tied to most-favored-nation pricing deals, the EU wants Buy-European supply, and 480 billion dollars of reshoring has been pledged.

The question to brief: where do you localise manufacturing versus accept tariffs, and which markets do you concede on price to protect the rest?

PAYERS & HEALTH SYSTEMS

Can you absorb accelerating cures and shrinking public health budgets at once?

The shift: the first oral GLP-1 is approved, CMS built a Medicare GLP-1 Bridge, gene therapies expand to children, while aid and public finance fall.

The question to brief: how do you cover a widening pipeline of expensive therapies while the fiscal base for health tightens?

INVESTORS & CAPITAL

Are you priced for a fragmented, sovereign-supply health market?

The shift: tariffs and pricing deals reset pharma margins, gene-editing data and AI drug design lift platform bets, and AI-care vendors attract strategic capital.

The question to brief: which assets are hedged against tariff and price risk, and which platforms turn the discovery wave into durable returns?

POLICY & GOVERNMENT AFFAIRS

What is your posture as the multilateral health order thins?

The shift: the US completed its WHO exit, the budget was cut 21 percent, China stepped into the assessed-contribution gap, and Africa talks health sovereignty.

The question to brief: do you align with a bloc, invest in regional capacity, or work to rebuild multilateral institutions, and on what timeline?

Themes

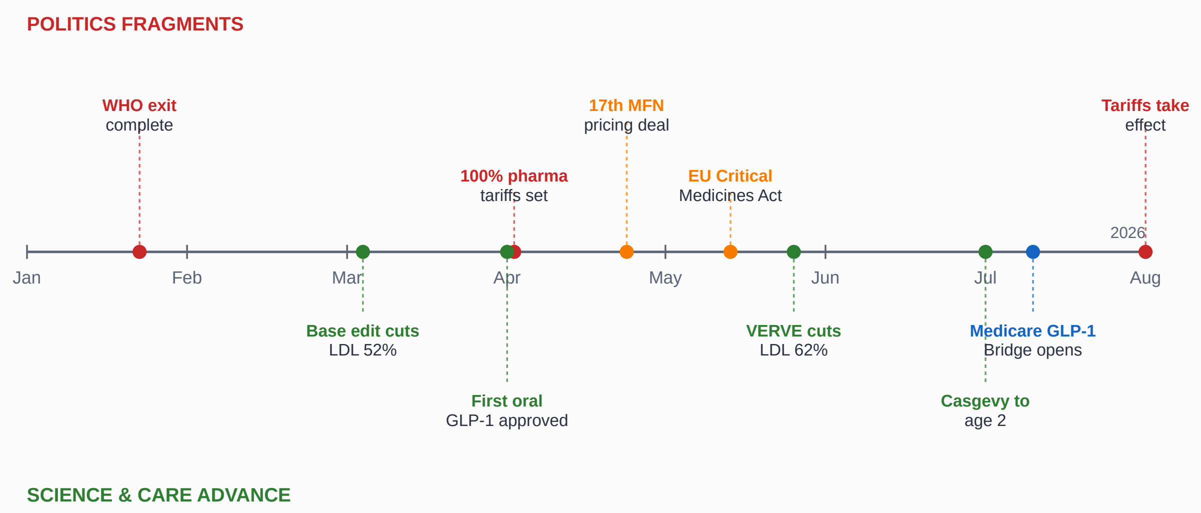
*The cycle's signals are organised into four themes, ranked by impact on near-term decisions across the sector. **Immediate**: shapes supply, pricing and policy decisions now. **Near-Term**: shapes position over the next year or two. **Longer-Range**: a multi-year shift to track each cycle.*

1. Medicine Becomes Industrial Policy: tariffs, price controls and the scramble for supply security

IMMEDIATE

Medicine became an instrument of statecraft this cycle. The US imposed a 100 percent tariff on patented pharmaceuticals and tied relief to most-favored-nation pricing agreements; the EU reached a Critical Medicines Act with a Buy-European procurement tilt; and both sides moved to reshore manufacturing and reduce dependence on foreign, largely Chinese, ingredients. Supply and price, once left to markets, are now levers of national power, and the scramble for supply security is on.

The summer medicine chose sides: a 2026 timeline



The cycle in one view: political fragmentation above the line, scientific and care advances below, running in parallel through 2026. Analyst construction from the sourced events.

- On 2 April 2026 the US imposed a 100 percent Section 232 tariff on patented pharmaceuticals, effective 31 July 2026 for seventeen large firms and 29 September for others, with generics exempt and thirteen pricing-agreement companies granted zero through January 2029. Ropes & Gray (7 April 2026).
- An additional 100 percent duty applies to patented drugs and their active ingredients from 31 July 2026, with generics, biosimilars and designated orphan drugs exempt. Arnall Golden Gregory (13 April 2026).
- An April 2026 executive order ties drug tariffs to pricing behaviour, offering zero tariffs to companies that enter most-favored-nation agreements and escalating 15 to 100 percent tariffs on those that decline. Sidley Austin (22 April 2026).
- The Regeneron most-favored-nation agreement was the seventeenth such deal, covering manufacturers representing 86 percent of the branded market, cutting Praluent from 537 to 225 dollars and committing 27 billion dollars of US investment. The White House (23 April 2026).
- The White House projection of more than 500 billion dollars in savings from most-favored-nation pricing rests on speculative modelling that drugmakers' own shareholder statements contradict.

Forbes (7 May 2026).

- Section 232 pharmaceutical tariffs function as a tax on imports and could raise US drug prices by roughly 30 percent, making them an ineffective affordability tool. American Action Forum (6 April 2026).
- The EMA welcomed the 12 May 2026 provisional agreement on the Critical Medicines Act as a milestone combining regulatory and industrial-policy measures to strengthen supply and production of critical medicines in Europe. European Medicines Agency (12 May 2026).
- The Critical Medicines Act introduces EU-preference procurement, designates strategic manufacturing projects and lowers the collaborative-procurement threshold from nine member states to five. McCann FitzGerald (15 May 2026); the deal also expands scope to orphan medicines (RAPS, 14 May 2026).
- Under tariff pressure, manufacturers have committed over 480 billion dollars to US production, spanning 22 new sites and roughly 44,000 jobs over four to ten years. Pharmaceutical Commerce (8 July 2026).
- India's imports from China account for 70 percent or more of its active-ingredient imports, with paracetamol and penicillin derivatives above 90 percent, despite production-linked incentive gains. IndraStra Global (5 June 2026).

Counter-argument

The tariff may be a negotiating lever, not a wall. Its relief is explicitly tied to pricing deals, seventeen of which were signed within weeks, and the exemptions for generics and orphan drugs blunt its edge; a serious analysis argues the headline rate overstates the real-world burden. If the 100 percent duty is serially waived rather than collected, the supply-security scramble slows and the status quo largely holds. The falsifiable test is whether the tariff is actually collected at scale in 2027. Actors to watch: US Commerce, the pharma majors, and the WTO.

Weak signals to watch

- WEAK SIGNAL Tariffs and Buy-European rules could bifurcate the global pharma supply chain into rival blocs. Would gain weight if a third major economy adopts local-content procurement for medicines.
- WEAK SIGNAL Reshoring pledges could stall at the announcement stage. Would gain weight if a flagship US or EU plant misses its build timeline or is quietly cancelled.

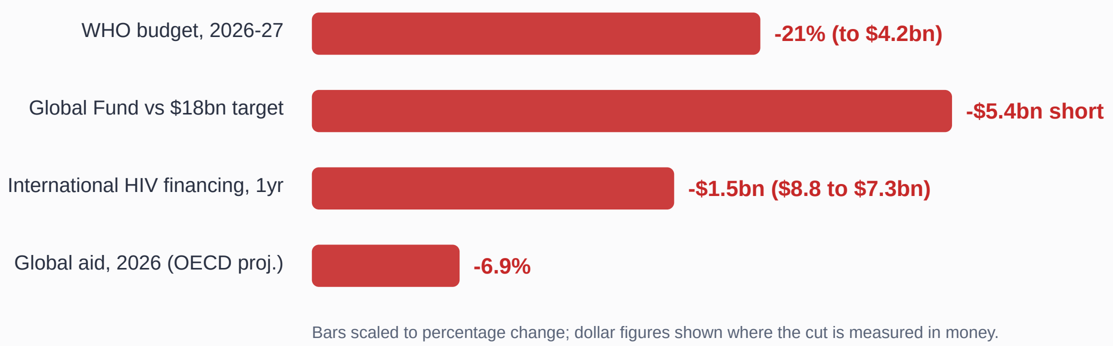
Decision link: Strategic Implications 1 and 2.

2. The Global Health Commons Retreats: the US exit, collapsing finance and the scramble to fill the vacuum

IMMEDIATE

The multilateral health order the US built is thinning. Washington completed its withdrawal from the WHO, cut aid, and withheld appropriated funds, and the institution's budget and staff shrank in response. Replenishments for the Global Fund and Gavi came in short, HIV financing fell sharply, and the resulting vacuum is being contested rather than filled: China has become the WHO's largest assessed contributor, and African governments are talking of health sovereignty. The retreat is a governance and financing shock whose full effects will land on the poorest first.

The money leaving global health, 2026



The retreat in four measures. Source: Think Global Health (WHO budget), Health Policy Watch (Global Fund), UNAIDS (HIV financing), OECD via ScienceBlog (global aid).

- The US stated its WHO withdrawal was completed on 22 January 2026, and the organisation shed staff amid the resulting shortfall. KFF (17 March 2026).
- The WHO's 2026-27 budget was cut 21 percent to 4.2 billion dollars and its workforce fell roughly 10 percent to 8,569, with outstanding assessed contributions rising to 184 million dollars, three-quarters owed by the US. Think Global Health (29 May 2026).
- International HIV financing fell 1.5 billion dollars in a year, from 8.8 billion in 2024 to 7.3 billion in 2025, and without action more than three million people could be newly infected with HIV by 2030. UNAIDS (27 July 2026).
- The Global Fund's eighth replenishment secured 12.64 billion dollars, of which 10.78 billion was allocated to countries for 2026-2028. The Global Fund (18 February 2026).
- That total fell 5.36 billion dollars short of the 18 billion dollar target, as France cut its pledge 58 percent and the Gates Foundation pledged 912 million dollars. Health Policy Watch (11 March 2026).
- Gavi's 2026-2030 replenishment secured more than 9 billion dollars against an 11.9 billion dollar target, while the US withheld the 300 million dollars Congress appropriated for FY2025 and FY2026. KFF (4 May 2026).
- Combined US and Global Fund reductions across 29 memorandum-of-understanding countries are estimated at 4.3 billion dollars, a 24 percent drop, with US cuts alone reaching 3.3 billion by 2029.

KFF (10 June 2026).

- The OECD projects a 6.9 percent decline in overall global aid in 2026, with bilateral health and population aid falling 29 to 46 percent between 2024 and 2026. OECD via ScienceBlog (23 July 2026).
- Official development assistance to Africa fell from about 26 billion dollars in 2021 to around 13 billion in 2025, and only three of 54 African nations meet the pledge to spend 15 percent of budgets on health, prompting talk of health sovereignty. PBS NewsHour (18 May 2026).
- China is now the WHO's top assessed contributor at just over 20 percent of the budget, about 138 million dollars a year, yet its voluntary donations were only 3 million dollars in 2025. The Star (6 June 2026).

Counter-argument

A leaner, multipolar system could prove more resilient than the one it replaces. Donor dependence bred fragility, and the shock is prompting African governments toward domestic financing and regional manufacturing, while China and others step into assessed-contribution gaps. If the vacuum catalyses self-reliance rather than collapse, the medium-term order could be less US-dependent but more robust. The falsifiable test is whether recipient-country domestic health spending rises measurably by 2028. Actors to watch: the Africa CDC, China, the Gates Foundation, and the EU.

Weak signals to watch

- **WEAK SIGNAL** A major disease resurgence could follow the financing cuts within the horizon. Would gain weight if a documented rise in HIV, TB or measles cases is attributed to 2025-26 funding withdrawals.
- **WEAK SIGNAL** A rival, China-anchored global health institution could emerge outside the WHO. Would gain weight if a new multilateral health financing body launches without US participation.

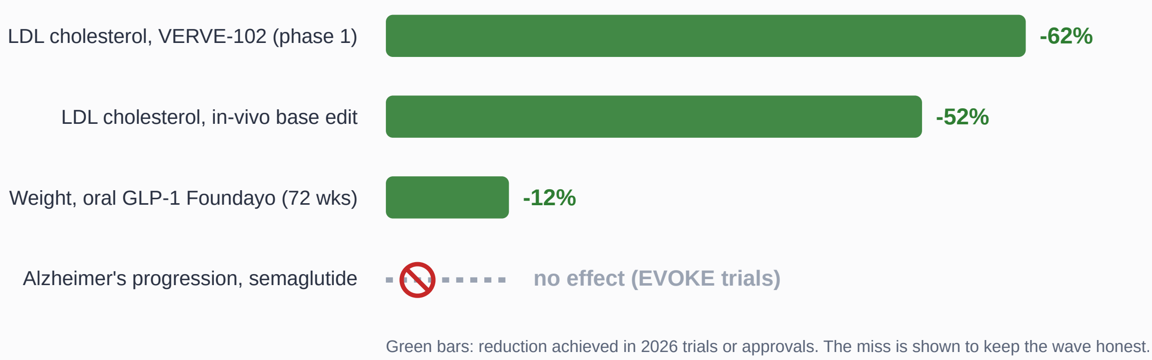
Decision link: Strategic Implications 2 and 4.

3. Discovery and Access Accelerate: AI-designed drugs, gene editing and the widening GLP-1 wave

NEAR-TERM

The science ran ahead of the politics. In-vivo gene editing moved from promise to phase-1 proof, cutting cholesterol in a single dose; the first oral GLP-1 was approved and Medicare built a coverage bridge; the first CRISPR therapy expanded to young children; and regulators set out how AI can be used to design and develop drugs. The discovery wave is real and broad, but it reopens the oldest question in health: as the cures multiply and the price tags rise, who actually gets them.

What the science delivered, and did not, in 2026



Efficacy from 2026 trials and approvals, with the notable miss kept in view. Source: Nature Medicine and STAT (LDL editing), Healio and the FDA (oral GLP-1), Nature (Alzheimer's).

- A single intravenous dose of an in-vivo base editor produced sustained reductions of 74 percent in PCSK9 and 52 percent in LDL cholesterol at 24 weeks, with no serious adverse events. Nature Medicine (3 March 2026).
- A rival in-vivo base-editing therapy, backed by Eli Lilly, cut cholesterol 62 percent at its high dose in a phase 1 study with no treatment-related serious adverse events. STAT News (25 May 2026).
- The FDA expanded Casgevy, the first approved CRISPR gene therapy, to children as young as two, making roughly 5,500 additional US children eligible for the one-time treatment. Vertex (1 July 2026).
- The FDA approved orforglipron, marketed as Foundayo, the first once-daily oral GLP-1, taken without food or water restrictions and producing about 12 percent weight loss at 72 weeks. Healio (1 April 2026); the FDA approval letter is dated 1 April 2026.
- CMS's BALANCE model negotiates GLP-1 prices directly with manufacturers, covering Mounjaro, Ozempic, Wegovy, Zepbound and Foundayo across Medicaid and Medicare. CMS (22 June 2026).
- The Medicare GLP-1 Bridge runs from 1 July 2026 to 31 December 2027 at a 50-dollar copay, with manufacturers supplying drugs at 245 dollars a month, after CMS delayed the broader BALANCE model on 21 April 2026. KFF (11 May 2026).
- The FDA and EMA jointly published Guiding Principles of Good AI Practice in Drug Development in January 2026, setting out ten principles for using AI to expedite drug development. US Food and Drug Administration (current as of 1 May 2026).

- The FDA increasingly grants breakthrough designation to AI tools that solve problems clinicians cannot, such as detecting multiple cancers from one image or predicting mortality risk. STAT News (2 April 2026).
- Conventional drug development takes about a decade and more than a billion dollars, with roughly 90 percent of candidates failing, the barrier AI-designed drugs aim to lower as approval is argued to be near. Bloomberg Law (18 May 2026).
- The science does not always deliver: in the phase 3 EVOKE trials the GLP-1 semaglutide did not slow clinical progression of Alzheimer's disease. Nature (31 March 2026).

Counter-argument

Acceleration in the lab is not access at the bedside. A 52 percent LDL cut in six patients is a phase-1 signal, not an approved therapy; gene therapies remain priced beyond most systems; and the very coverage fights around GLP-1s, delayed and bridged rather than settled, show how hard financing the new medicine is even in rich countries. If payers cannot absorb the pipeline, discovery widens the gap between what is possible and what is provided. The falsifiable test is whether any 2026 breakthrough reaches broad reimbursed use by 2028. Actors to watch: CMS, HTA bodies, and the gene-therapy developers.

Weak signals to watch

- WEAK SIGNAL An AI-designed drug could reach the market within the horizon. Would gain weight if the FDA accepts a marketing application for a molecule with an AI-generated design as its origin story.
- WEAK SIGNAL Gene editing could shift from rare disease to common chronic conditions. Would gain weight if an in-vivo editor for high cholesterol enters phase 3 with a broad eligible population.

Decision link: Strategic Implications 3 and 1.

4. Digital Health Goes Ambient: AI scribes, agentic workflows and the machine in the consulting room

LONGER-RANGE

Artificial intelligence moved into the consulting room. Ambient scribes crossed from pilots to routine use, agentic workflows arrived on the roadmaps of the largest vendors, and roughly three-quarters of US health systems now use or plan to use an AI platform. The productivity case is increasingly evidenced, in documentation time, clinician burnout and eye contact. The safety and evidence base is the lagging variable: recalls, sparse published trials and unresolved questions of bias and oversight sit alongside the adoption curve, not behind it.

- Across 2.33 million assisted encounters, ambient AI scribe adoption rose from 2.7 percent to about 31 percent of outpatient visits while semantic agreement stayed near 88 percent. Frontiers in Digital Health (6 July 2026).
- A controlled time-motion study found an ambient AI scribe cut documentation time 15 percent and increased clinician eye contact by about 11 percent of consultation time. JMIR Medical Informatics (31 March 2026).
- Ambient scribe deployments reported a 21 percent reduction in burnout at Mass General Brigham after 84 days and a 27 percent reduction in note time at Intermountain Health. American Hospital Association (14 April 2026).
- About 75 percent of US health systems use or plan to use an AI platform in 2026, and more than half able to quantify return reported at least a doubling on deployed AI. Fierce Healthcare (24 March 2026).
- Epic previewed Agent Factory, a visual builder letting health systems create and deploy their own agentic AI agents under local policies. Fierce Healthcare (10 March 2026).
- The ambient-scribe vendor Abridge partnered with Nvidia on a clinical-conversation foundation model and took a strategic investment from Eli Lilly as it expands beyond documentation. Healthcare Dive (12 June 2026).
- The share of US adults using AI chatbots for health questions doubled from 16 to 32 percent in a year, according to Rock Health's eleventh consumer survey. Rock Health (23 March 2026).
- The FDA has authorised about 1,450 AI-enabled medical devices, most Class II cleared via the 510(k) pathway, with predetermined change-control plans now used for updates. Congressional Research Service (10 June 2026).
- Among 903 AI-enabled devices, 43, or 4.8 percent, were recalled, a median 458 days after authorisation, with higher recall risk where clinical study information was not published. JAMA Network Open (11 June 2026).
- Physicians rated 88 of 100 unedited AI-generated hospital-course summaries as having no harm potential, with only one judged likely to cause moderate harm, evidence that is reassuring but narrow. JAMA Network Open (8 May 2026).

Counter-argument

Adoption may be outrunning the evidence rather than resting on it. Much of the productivity case comes from vendor-reported deployments and single-site studies, the published trial base is thin, and a measurable share of AI devices has been recalled. If a high-profile safety failure or a bias scandal lands before the evidence base matures, trust and reimbursement could tighten sharply and slow the curve. The falsifiable test is whether peer-reviewed, multi-site outcome evidence for ambient and agentic AI accumulates faster than adverse events through 2027. Actors to watch: the FDA, JAMA and NEJM, and the health-system CMIOs.

Weak signals to watch

- **WEAK SIGNAL** A serious AI safety event could trigger a regulatory and reimbursement snap-back. Would gain weight if a patient-harm case is attributed to an ambient or agentic AI tool and reaches national headlines.
- **WEAK SIGNAL** Agentic AI could move from documentation into clinical decision-making faster than oversight allows. Would gain weight if a health system deploys autonomous AI agents in triage or ordering at scale.

Decision link: Strategic Implication 4.

Strategic Implications

Four decisions this cycle puts to leaders across pharma, payers, investors and policy, each with a posture, an owner and the themes it draws on.

SI 1: Decide how to build tariff and price resilience into supply and pricing

Medicine is now industrial policy: a 100 percent US tariff tied to most-favored-nation pricing, an EU Buy-European tilt, and 480 billion dollars of reshoring pledges. Firms cannot both absorb tariffs and hold price everywhere. Decide explicitly where to localise manufacturing against tariff exposure, which markets to concede on price to protect the whole, and how to sequence reshoring so pledges become operating capacity rather than announcements. Treat sovereignty as a cost of doing business in the largest markets, priced in rather than wished away.

Action: this cycle, set a board-level supply-and-pricing map, by market, of where to localise, where to accept tariffs, and where to sign pricing deals.

Who gains: the manufacturers that localise selectively and early, and the contract manufacturers and API producers inside favoured blocs.

DECIDE

Draws on Themes 1 and 3. Owner: Pharma and biotech boards, Chief Supply and Commercial officers.

SI 2: Prepare for a multipolar health order and a financing vacuum

The US exited the WHO, the budget was cut, replenishments came short, and HIV financing fell while China stepped in and Africa turned to health sovereignty. Funders, ministries and multilateral leaders should prepare for a system that is less US-centred and more regional. Prepare domestic-financing and regional-manufacturing plans in exposed countries, and, for institutions, a leaner mandate and a broader donor base. If the vacuum is filled deliberately, the successor order can be more resilient than the one it replaces.

Action: within two cycles, exposed governments and funders should publish a domestic-financing and regional-capacity plan for the programmes most reliant on withdrawn US support.

Who gains: the regions that build domestic financing and manufacturing early, and the institutions that broaden their donor base beyond a single patron.

PREPARE

Draws on Theme 2. Owner: Global-health funders, health ministries, multilateral leadership.

SI 3: Prepare to finance an accelerating pipeline against a tightening fiscal base

The cures are arriving faster, in-vivo editing, an oral GLP-1, expanded gene therapy, even as public budgets fall and coverage fights, such as the delayed and bridged GLP-1 model, show how hard financing them is. Payers, HTA bodies and access strategists should prepare outcome-based and instalment financing, and a triage framework for which therapies to cover, for whom, and when. The risk is a widening gap between what medicine can do and what systems will pay for; the opportunity is to be the payer or provider that solves access before rivals do.

Action: this cycle, stand up an access-and-financing framework for high-cost therapies, with outcome-based contracting and explicit coverage-sequencing criteria.

> **Who gains:** the payers that master novel financing, and the developers that design for reimbursability, not just efficacy.

PREPARE

Draws on Theme 3. Owner: Payers, HTA bodies, R&D and access strategy.

SI 4: Monitor ambient and agentic AI, and gate adoption on safety and evidence

AI is in the consulting room now: scribe use scaled to a third of visits, three-quarters of systems adopting, and agentic tools on the roadmap, while 4.8 percent of AI devices have been recalled. Health-system leaders and regulators should capture the documented productivity gains while gating deployment on published, multi-site safety and outcome evidence, and on clear oversight for anything that touches decisions. Do not let agentic AI move into triage or ordering ahead of the evidence and the accountability model. Monitor for the first serious adverse event, which could reset trust and reimbursement.

Action: monitor for peer-reviewed multi-site AI outcome evidence and for adverse events, and adopt ambient AI now while holding agentic clinical AI behind an evidence-and-oversight gate.

> **Who gains:** the systems that capture documentation gains without owning the first safety scandal, and the vendors that publish real evidence.

MONITOR

Draws on Theme 4. Owner: Health-system executives, CMIOs, regulators.

Scenario Matrix

Four operating environments for health to 2030, generated by crossing two axes the evidence does not yet decide: whether the medicine order re-integrates or stays fragmented into national blocs, and whether the accelerating science reaches broadly or is captured narrowly by those who can pay. The matrix is a planning tool, not a forecast; the value sits in the indicators that would tip the system from one cell to another.

MEDICINE ORDER (RE-INTEGRATES AT LEFT, STAYS FRAGMENTED AT RIGHT)

RE-INTEGRATES

FRAGMENTED

ACCESS TO THE SCIENCE (BROADLY SHARED AT TOP, NARROWLY CAPTURED AT BOTTOM)

The Shared Cure

The best pairing: the tariff regime is unwound through pricing deals, the US re-engages with multilateral institutions, and the discovery wave reaches broadly through coverage bridges and tiered pricing. Fragmentation proves to be a phase, not a destination, and the new medicine diffuses. Supply is more resilient than in 2024 and access is wider.

Indicators: tariffs are waived or wound down; the US resumes WHO or Gavi funding; gene therapies and GLP-1s reach broad reimbursed use.

Managed Fragmentation

The blocs harden but each provisions its own population well. Tariffs and Buy-European rules stick, supply chains split into US, EU and Asian spheres, and reshoring succeeds, at higher cost. Within each bloc access holds; between them, duplication and price divergence grow. A more expensive, more resilient, more divided world.

Indicators: tariffs are collected; reshoring plants open; a third economy adopts local-content rules; within-bloc coverage stays broad.

Recovery, Unevenly

The order re-integrates but access stays narrow. Multilateral institutions stabilise and trade normalises, yet the cost of gene therapies and novel drugs, and thin public budgets, keep the new medicine confined to the wealthy. The politics heal faster than the financing, and the abundance-scarcity gap persists inside an ostensibly reconnected system.

Indicators: tariffs ease and aid partially recovers, but HTA rejections rise, gene-therapy list prices hold, and low-income access lags.

BROAD ACCESS

Sovereign Silos

The worst pairing: fragmentation hardens and access narrows together. Tariffs and sovereignty raise prices, the financing vacuum deepens as aid keeps falling, disease resurges where programmes collapsed, and the new cures reach only the richest markets. Medicine splits permanently into a world of abundance and a world of retreat.

Indicators: tariffs entrench; aid keeps falling; a documented disease resurgence; gene and GLP-1 therapies stay confined to high-income systems.

NARROW ACCESS

What We Are Not Planning For

Four scenarios held out of the plan because the evidence does not yet justify resourcing against them. Each carries the reinstatement trigger that would change that judgement.

Exclusion 1: A full US return to the multilateral health system within the horizon

The WHO withdrawal is complete and aid has been cut across dozens of countries. A partial re-engagement is possible, but a wholesale return to pre-2025 US funding and leadership within the horizon is not the base case. We are not planning for a full US multilateral restoration, while treating targeted re-engagement as a live upside.

Reinstatement trigger: *a US administration resumes WHO membership and restores appropriated Gavi and Global Fund contributions.*

Exclusion 2: Tariffs fully reshoring the pharmaceutical supply chain by 2030

Reshoring pledges are large but plants take years, and ingredient dependence on China runs deep. We are not planning for a fully reshored US or EU supply chain within the horizon; the realistic outcome is partial, costly localisation of the most critical products.

Reinstatement trigger: *multiple flagship API and finished-dose plants reach commercial operation and materially cut import reliance.*

Exclusion 3: An AI-designed drug reaching market approval within the horizon

Regulators have set AI-in-development principles and observers argue approval is not far off, but designing a molecule is not approving a therapy, and trials take years. We are not planning for a fully AI-designed drug to gain approval within the near horizon, while tracking it as a watch item.

Reinstatement trigger: *the FDA accepts a marketing application whose lead molecule was AI-generated, with pivotal trial data.*

Exclusion 4: GLP-1 drugs proving a broad panacea beyond metabolic disease

The GLP-1 wave is real, but the Alzheimer's trials failed, and not every adjacent indication will land. We are not planning for GLP-1s to become a general-purpose therapy across neurodegenerative and other conditions within the horizon.

Reinstatement trigger: *a well-powered phase 3 shows GLP-1 benefit in a major non-metabolic indication such as dementia or addiction.*

Discussion Points

1. If medicine is now industrial policy, where do we localise manufacturing against tariff exposure, and which markets do we concede on price to protect the whole?
2. **For funders and ministries:** as the US-built multilateral order thins, do we align with a bloc, invest in regional capacity, or work to rebuild shared institutions, and on what timeline?
3. If the cures are arriving faster than budgets can absorb them, what is our framework for deciding which high-cost therapies to cover, for whom, and when?
4. **For health-system leaders:** how do we capture the documented productivity gains from ambient AI while holding agentic clinical AI behind a safety-and-evidence gate?
5. If the science is converging while the politics fragment, where is our organisation exposed to the gap between what medicine can do and what our system will provision?

Source Confidence Register

42 verified sources across four themes. Tier 1 (governments, regulators, multilateral bodies, peer-reviewed primary): 11. Tier 2 (institutional research, quality journalism, peer-reviewed journals, professional societies, law-firm and consultancy analyses): 23. Tier 3 (specialist trade and quality press, company primary): 8. All 42 sources fall within the 180-day recency window back from the 6 August 2026 cycle; 34 of 42 (81%) sit at Tier 1 or 2, reflecting a policy-and-science cycle anchored on regulators, multilateral institutions and peer-reviewed evidence. Non-English source markets and jurisdictions (EU, China, India, Africa) are represented. The source set was frozen at 2026-08-06. Detect → Assess → Decide → Act.

Theme 1: Medicine Becomes Industrial Policy

Source	Tier	Date	Key claim
The White House	T1	23 Apr 2026	The Regeneron MFN agreement is the 17th such deal, covering manufacturers representing 86% of the branded drug market; Regeneron will cut Praluent from $537 to $225 via TrumpRx and commit $27 billion in US investment by 2029.
European Medicines Agency (EMA)	T1	12 May 2026	EMA endorsed the 12 May 2026 provisional agreement on the Critical Medicines Act as a milestone combining regulatory and industrial-policy measures to strengthen the availability, supply and production of critical medicines in Europe.
American Action Forum	T2	6 Apr 2026	Section 232 pharmaceutical tariffs function as a tax on imports and could raise domestic drug prices by roughly 30 percent, making them an ineffective instrument for addressing drug affordability.
Ropes & Gray LLP	T2	7 Apr 2026	Trump's 2 April 2026 proclamation imposes 100% Section 232 tariffs on patented pharmaceuticals effective 31 July 2026 for 17 large firms and 29 September 2026 for others, with generics exempt and 13 MFN-agreement companies granted 0% through 20 January 2029.
Arnall Golden Gregory LLP	T2	13 Apr 2026	Under a White House proclamation issued 6 April 2026, an additional 100% Section 232 duty applies to patented pharmaceuticals and their active ingredients beginning 31 July 2026, with generics, biosimilars and designated orphan drugs exempt.

Source	Tier	Date	Key claim
Sidley Austin LLP	T2	22 Apr 2026	An April 2026 executive order ties drug tariffs to pricing behaviour, offering 0% tariffs to companies that enter MFN pricing agreements while imposing escalating 15-100% tariffs on those that decline.
Forbes	T2	7 May 2026	The White House's projected $500+ billion in savings from MFN drug pricing rests on speculative modelling and unrealistic assumptions, contradicted by drugmakers' own shareholder statements anticipating minimal impact.
McCann FitzGerald LLP	T2	15 May 2026	The 12 May 2026 provisional CMA agreement introduces EU-preference procurement, designates strategic manufacturing projects and lowers the collaborative procurement threshold from 9 to 5 Member States.
Regulatory Affairs Professionals Society (RAPS)	T3	14 May 2026	EU Parliament and Council reached a provisional Critical Medicines Act agreement establishing procurement-resilience requirements, an EU-preference approach to incentivise local production and expanded scope covering orphan medicinal products.
IndraStra Global	T3	5 Jun 2026	India's imports from China accounted for 70 percent or more of total API imports across FY2023-24 and 2024-25, with categories such as paracetamol and penicillin derivatives exceeding 90-95 percent Chinese dependence despite PLI-scheme gains.
Pharmaceutical Commerce	T3	8 Jul 2026	Following tariff pressure, pharmaceutical manufacturers have committed over $480 billion to US-based production, spanning 22 new manufacturing sites and roughly 44,000 new jobs over the next 4-10 years.

Theme 2: The Global Health Commons Retreats

Source	Tier	Date	Key claim
The Global Fund	T1	18 Feb 2026	The Global Fund's Eighth Replenishment secured US$12.64 billion, of which US$10.78 billion was allocated to countries for 2026-2028 implementation.
UNAIDS	T1	27 Jul 2026	International HIV financing declined by US$1.5 billion in one year, from US$8.8 billion in 2024 to US$7.3 billion in 2025, and without urgent action more than 3 million people could become newly infected with HIV by 2030.
Health Policy Watch	T2	11 Mar 2026	The Global Fund's US$12.64 billion fell US$5.36 billion short of its US$18 billion target, with France cutting its pledge 58% and the Gates Foundation pledging US$912 million.
KFF (Kaiser Family Foundation)	T2	17 Mar 2026	The administration stated its WHO withdrawal was completed on January 22, 2026, and WHO eliminated almost 3,000 positions (22% of its staff) amid the resulting funding shortfall.
KFF (Kaiser Family Foundation)	T2	4 May 2026	Gavi's 2026-2030 replenishment secured pledges of more than $9 billion against an $11.9 billion target, while the US administration withheld the $300 million Congress appropriated for both FY2025 and FY2026.
PBS NewsHour	T2	18 May 2026	Official development assistance to Africa fell from about $26 billion in 2021 to around $13 billion in 2025, and only three of 54 African nations meet the 2001 pledge to spend 15% of national budgets on health.
Think Global Health (Council on Foreign Relations)	T2	29 May 2026	WHO's 2026-27 budget was slashed 21% to $4.2 billion and its workforce fell roughly 10% to 8,569 employees, with outstanding assessed contributions rising to $184 million, 75% of it owed by the United States.
KFF (Kaiser Family Foundation)	T2	10 Jun 2026	Combined U.S. and Global Fund reductions across 29 MOU countries are estimated at $4.3 billion, a 24% drop, with U.S. cuts alone reaching $3.3 billion (29%) by 2029.

Source	Tier	Date	Key claim
The Star (Malaysia)	T3	6 Jun 2026	China is now WHO's top assessed contributor at just over 20% of the budget (about $138 million a year, up 57% from 2024-25), yet its voluntary donations were only $3 million in 2025, leaving it 10th overall.
ScienceBlog	T3	23 Jul 2026	The OECD projects a 6.9% decline in overall global aid in 2026, with bilateral aid for health and population programs falling 29% to 46% between 2024 and 2026.

Theme 3: Discovery and Access Accelerate

Source	Tier	Date	Key claim
Nature Medicine	T1	3 Mar 2026	A single intravenous dose of the in vivo base editor YOLT-101 produced sustained reductions of 74.4% in PCSK9 and 52.3% in LDL-C at 24 weeks in the highest-dose cohort, with no serious adverse events.
U.S. Food and Drug Administration	T1	1 Apr 2026	The FDA approved Foundayo (orforglipron) tablets effective 1 April 2026 to reduce and maintain weight loss in adults with obesity or overweight with at least one comorbidity.
U.S. Food and Drug Administration	T1	1 May 2026	The FDA (CDER, CBER) and the EMA jointly published Guiding Principles of Good AI Practice in Drug Development in January 2026, setting out 10 guiding principles for using AI to expedite drug and biological product development.
Centers for Medicare & Medicaid Services (CMS)	T1	22 Jun 2026	CMS's BALANCE model negotiates GLP-1 prices directly with manufacturers, with Medicaid coverage from May 2026 and Medicare access via the GLP-1 Bridge from July 2026, covering Mounjaro, Ozempic, Rybelsus, Wegovy, Zepbound and Foundayo.
Nature	T2	31 Mar 2026	In the phase 3 EVOKE and EVOKE+ trials, the GLP-1 receptor agonist semaglutide did not slow clinical progression of Alzheimer's disease, despite earlier observational promise.

Source	Tier	Date	Key claim
Healio	T2	1 Apr 2026	The FDA approved orforglipron (Foundayo), a once-daily oral GLP-1 from Eli Lilly taken without food or water restrictions, which produced about 12.4% weight loss at the highest dose at 72 weeks.
STAT News	T2	2 Apr 2026	The FDA increasingly grants breakthrough designation to AI tools that solve problems physicians cannot, such as detecting multiple cancers from a single image or predicting risk of death.
KFF (Kaiser Family Foundation)	T2	11 May 2026	The Medicare GLP-1 Bridge runs 1 July 2026 to 31 December 2027, covering select obesity GLP-1s at a $50 monthly copay with manufacturers supplying drugs at $245 per month, after CMS delayed the BALANCE model on 21 April 2026.
Bloomberg Law	T2	18 May 2026	Conventional drug development takes about a decade and more than $1 billion on average, with roughly 90% of candidates never reaching approval, a barrier AI-designed drugs aim to lower.
STAT News	T2	25 May 2026	A high dose of Verve's in vivo base-editing therapy VERVE-102 reduced cholesterol levels by 62% in Phase 1 participants, with no treatment-related serious adverse events.
Vertex Pharmaceuticals	T3	1 Jul 2026	The FDA expanded CASGEVY, the first approved CRISPR gene therapy, to children as young as 2, making approximately 5,500 additional US children eligible for the one-time treatment.

Theme 4: Digital Health Goes Ambient

Source	Tier	Date	Key claim
JAMA Network Open	T1	8 May 2026	Physicians rated 88 of 100 unedited AI-generated hospital course summaries (88.0%) as having no harm potential, with only one judged likely to cause moderate harm.

Source	Tier	Date	Key claim
Congressional Research Service	T1	10 Jun 2026	The FDA has authorized approximately 1,450 AI-enabled medical devices since 1995, most Class II cleared via the 510(k) pathway, with Predetermined Change Control Plans now used for postmarket updates.
JAMA Network Open	T1	11 Jun 2026	Among 903 AI-enabled medical devices, 43 (4.8%) were recalled, a median 458 days from authorization to recall, with higher recall risk for devices lacking published clinical study information.
Fierce Healthcare	T2	10 Mar 2026	Epic previewed Agent Factory, a visual builder letting health systems create, equip with local policies, and deploy their own agentic AI agents.
Rock Health	T2	23 Mar 2026	The share of US adults using AI chatbots for health questions doubled from 16% to 32% in one year, per Rock Health's 11th annual Consumer Adoption survey.
Fierce Healthcare	T2	24 Mar 2026	About 75% of US health systems use or plan to use an AI platform in 2026, and more than half able to quantify ROI reported at least a 2x return on deployed AI.
JMIR Medical Informatics	T2	31 Mar 2026	An ambient AI scribe cut documentation time by 15.0% (5.3 to 4.5 minutes per consultation) and increased clinician eye contact by 10.6% (69.6% to 77.1% of consultation time).
Frontiers in Digital Health	T2	6 Jul 2026	Across 2.33 million assisted encounters, ambient AI scribe adoption rose from 2.7% to about 31% of all outpatient visits while semantic agreement stayed high at 87.4-89.2%.
American Hospital Association	T3	14 Apr 2026	Ambient AI scribe deployments reported a 21.2% reduction in burnout at Mass General Brigham after 84 days and a 27% reduction in time in notes at Intermountain Health with Dragon Copilot.

Source	Tier	Date	Key claim
Healthcare Dive	T3	12 Jun 2026	Ambient AI scribe vendor Abridge partnered with Nvidia to build a clinical-conversation foundation model and took a strategic investment from Eli Lilly as it expands beyond documentation.

Claim-Fidelity Appendix: Analyst inferences and editorial framing

This briefing carries analyst-generated interpretations that go beyond what any single source asserts. They are named here so a reader can trace the confidence line and disagree productively.

Analytical inferences carried by the body prose

- "Medicine chose sides" is an analyst frame linking four independently sourced threads, the tariff-and-pricing regime, the multilateral retreat, the discovery wave and ambient AI, into one pattern of fragmenting politics against converging science; no single source asserts the composite.
- The reading that fragmentation may be a phase rather than a destination, unwound through pricing deals and re-engagement, is an interpretation of the tariff-relief mechanism and historical precedent, and is contested in the Sovereign Silos scenario.
- The judgement that the successor global-health order could prove more resilient than the one it replaces is an analyst inference from the shift toward domestic financing and regional manufacturing; it is disclosed as contested in the same theme's counter-argument.
- The energy-after-1973 analogy is an interpretive overlay chosen to discipline the medicine-as-statecraft thesis, not a claim the sources make.
- Treating accelerating discovery as widening the access gap, rather than simply improving care, is an analyst inference from the coverage fights and the price of new therapies; the opposite reading is held in the counter-argument to Theme 3.

Editorial framing

The title "The Summer Medicine Chose Sides" is an editorial compression: it names the through-line that supply, price and access turned national this cycle even as the science globalised. As the second edition in this topic's Decision Intelligence series, this report grades the June cycle's six forces in the continuity ledger rather than setting a fresh baseline. Market-forecast and projection figures are directional, cited to a single provider each and flagged as such; where a number is load-bearing it is corroborated by a regulatory fact, an official statement or peer-reviewed evidence rather than resting on a forecast alone.

The Futures Wheel and the in-body figure SVGs are analyst constructions built from the sourced claims; they carry no separate source-register entry, so a claim or quantification flag on those elements is expected and disclosed here rather than being a provenance failure.